

BACK

140 mm

infarction and stroke, which may increase with dose or duration of use. Maxigesic[®] is contraindicated in patients with heart problems.

Hypertension:
NSAIDs may lead to onset of new hypertension or worsening of pre-existing hypertension and patients taking antihypertensive medicines with NSAIDs may have an impaired anti-hypertensive response. Caution is advised when prescribing Maxigesic[®] to patients with hypertension. Blood pressure should be monitored closely during initiation of treatment with Maxigesic[®] and at regular intervals thereafter.

Severe Skin Reactions
NSAIDs may very rarely cause serious cutaneous adverse events such as exfoliative dermatitis, toxic epidermal necrolysis (TEN) and Stevens-Johnson syndrome (SJS), which can be fatal and occur without warning. These serious adverse events are idiosyncratic and are independent of dose or duration of use. Patients should be advised of the signs and symptoms of serious skin reactions and to consult their doctor at the first appearance of a skin rash or any other sign of hypersensitivity.

Pre-existing asthma
Products containing ibuprofen should not be administered to patients with aspirin sensitive asthma and should be used with caution in patients with pre-existing asthma.

Ophthalmological effects
Adverse ophthalmological effects have been observed with NSAIDs; accordingly, patients who develop visual disturbances during treatment with ibuprofen should have an ophthalmological examination.

Combination use of ACE inhibitors or angiotensin receptor antagonists, anti-inflammatory drugs and thiazide diuretics
The use of an ACE inhibiting drug (ACE-inhibitor or angiotensin receptor antagonist), an anti-inflammatory drug (NSAID or COX-2 inhibitor) and thiazide diuretic at the same time increases the risk of renal impairment. This includes use in fixed-combination products containing more than one class of drug. Combined use of these medications should be accompanied by increased monitoring of serum creatinine, particularly at the institution of the combination. The combination of drugs from these three classes should be used with caution particularly in elderly patients.

Aseptic Meningitis
For products containing ibuprofen aseptic meningitis has been reported only rarely, usually but not always in patients with systemic lupus erythematosus (SLE) or other connective tissue disorders.

Masking Signs of Infection
As with other drugs of this class containing ibuprofen, by reducing fever this may mask the usual signs of infection.

Special Precautions
In order to avoid exacerbation of disease or adrenal insufficiency, patients who have been on prolonged corticosteroid therapy should have their therapy tapered slowly rather than discontinued abruptly when ibuprofen is added to the treatment program.

Use in pregnancy
Category C: drugs which, owing to their pharmacological effects, have caused or may be suspected of causing, harmful effects on the human foetus or neonate without causing malformations. These effects may be reversible.
NSAIDs inhibit prostaglandin synthesis and, when given during the latter part of pregnancy, may cause closure of the foetal ductus arteriosus, foetal renal impairment, inhibition of platelet aggregation and delay labour and birth.
Paracetamol has been taken by a large number of pregnant women and women of childbearing age without any proven increase in the frequency of malformations or other direct or indirect harmful effects on the foetus having been observed.

Further, there is inadequate information regarding the use of Maxigesic[®] in pregnancy. Therefore Maxigesic[®] should not be used during pregnancy or in patients planning to become pregnant.

Use in lactation
Maxigesic[®] is not recommended for nursing mothers.

INTERACTION WITH OTHER MEDICINES
Paracetamol
The following interactions have been noted:

- anticoagulant drugs (e.g.warfarin):dosage may require reduction if paracetamol and anticoagulants are taken for a prolonged period of time
- antiepileptic medications: the likelihood of toxicity may be increased by the concomitant use of enzyme inducing agents
- paracetamol absorption is increased by substances that increase gastric emptying, e.g. metoclopramide
- paracetamol absorption is decreased by substances that decrease gastric emptying
 - e.g. propantheline, antidepressants with anticholinergic properties, and narcotic analgesics
- paracetamol may increase chloramphenicol plasma concentrations
- hepatotoxic drugs or drugs that induce liver microsomal enzymes such as alcohol and anticonvulsant agents: the risk of paracetamol toxicity may be increased in patients receiving these drugs
- probenecid: may affect paracetamol excretion and plasma concentrations
- cholestyramine: reduces the absorption of paracetamol if given within 1 hour of paracetamol.
- isoniazid alone or combined with other drugs for tuberculosis: in patients receiving these drugs severe hepatotoxicity at therapeutic doses or moderate overdoses of paracetamol has been reported
- zidovudine and co-trimoxazole: severe hepatotoxicity has occurred after use of paracetamol in a patients taking these drugs.

Ibuprofen
The following interactions have been noted:

- anticoagulants, including warfarin – ibuprofen interferes with the stability of INR and may increase risk of severe bleeding and sometimes fatal haemorrhage, especially from the gastrointestinal tract. Ibuprofen should only be used in patients taking warfarin if absolutely necessary and they must be closely monitored.
- lithium: ibuprofen may decrease renal clearance and increase plasma concentration of lithium
- ACE inhibitors, beta-blockers and diuretics: ibuprofen may reduce the anti- hypertensive effect of these drugs and may cause natriuresis and hyperkalemia in patients under these treatments
- methotrexate: ibuprofen reduces methotrexate clearance
- cardiac glycosides: ibuprofen may increase the plasma levels of these drugs
- corticosteroids: the risk of ibuprofen-induced gastrointestinal bleeding may be increased with concomitant use of oral corticosteroids
- zidovudine: ibuprofen may prolong bleeding time in patients treated with this drug
- probenecid, antidiabetic medicine and phenytoin: these medicines may interact with ibuprofen

ADVERSE EFFECTS
Clinical trials with Maxigesic[®] have not indicated any other undesirable effects other than those for paracetamol alone or ibuprofen alone.

Side effects of paracetamol are rare and usually mild, although haematological reactions have been reported.

- Cutaneous hypersensitivity reactions including skin rashes, angioedema, Stevens Johnson Syndrome/Toxic Epidermal Necrolysis have been reported.

- Overdosage with paracetamol if left untreated can result in severe, sometimes fatal liver damage and rarely, acute renal tubular necrosis.
- Adverse effects with non-prescription (OTC) or short-term use ibuprofen are rare and may include:

- gastrointestinal – gastrointestinal bleeding, dyspepsia, heartburn, nausea, loss of appetite, stomach pain, diarrhoea
- central nervous system (CNS) – dizziness, fatigue,

headache, nervousness

- hypersensitivity reactions - skin rashes and itching. Rarely exfoliative dermatitis and epidermal necrolysis have been reported with ibuprofen.
- rare cases of photosensitivity
- cardiovascular – risks of myocardial infarct and stroke. These risks are minimal at Maxigesic[®] recommended maximum daily doses but increase with longer duration of treatment, and in the elderly. Fluid retention and in some cases oedema have been reported with all NSAIDs. These effects are rare at non-prescription doses.
- Allergic reactions such as skin rash, itching, swelling of the face or breathing difficulties may also occur. These are usually transient and reversible on cessation of treatment.

WARNING

Risk of GI Ulceration, Bleeding and Perforation with NSAID
Serious GI toxicity such as bleeding, ulceration and perforation can occur at any time, with or without warning symptoms, in patients treated with NSAID therapy. Although minor upper GI problems (e.g. dyspepsia) are common, usually developing early in therapy, prescribers should remain alert for ulceration and bleeding in patients treated with NSAIDs even in the absence of previous GI tract symptoms.

Studies to date have not identified any subset of patients not at risk of developing peptic ulceration and bleeding. Patients with prior history of serious GI events and other risk factors associated with peptic ulcer disease (e.g. alcoholism, smoking, and corticosteroid therapy) are at increased risk. Elderly or debilitated patients seem to tolerate ulceration or bleeding less than other individuals and account for most spontaneous reports for fatal GI events.

Allergy alert: Paracetamol may cause severe skin reactions. Symptoms may include skin reddening, blisters or rash. These could be signs of a serious condition. If these reactions occur, stop use and seek medical assistance right away.

WARNING:

This preparation contains PARACETAMOL.
Do not take any other paracetamol containing medicines at the same time.

DOSAGE AND ADMINISTRATION
Adults and Children over 12 years: The usual dosage is one to two tablets taken every six hours, as required, up to a maximum of eight tablets in 24 hours.
Children under 12 years: Maxigesic[®] is not recommended for children under 12 years. Do not use for more than 72 hours except on the advice of your doctor.

OVERDOSAGE
If an overdose is taken or suspected, ring the National Poisons Centre of Malaysia +604-657 0099 for the most up-to date treatment recommendations.
Symptoms
Paracetamol:
Symptoms of paracetamol overdosage in the first 24 hours are pallor, nausea, vomiting, anorexia and abdominal pain. Liver damage may become apparent 12 to 48 hours after ingestion. Abnormalities of glucose metabolism and metabolic acidosis may occur. In severe poisoning, hepatic failure may proceed to encephalopathy, coma and death. Acute renal failure with acute tubular necrosis may develop in the absence of severe liver damage. Cardiac arrhythmias have been reported. Liver damage is likely in adults who have taken 10 g or more of paracetamol, due to excess quantities of a toxic metabolite becoming irreversibly bound to liver tissue.
Ibuprofen
Symptoms include nausea, abdominal pain and vomiting, dizziness, convulsion and rarely, loss of consciousness. Clinical features of overdose with ibuprofen which may result are depression of the central nervous system and the respiratory system.

PRESENTATION AND STORAGE CONDITIONS
A white coloured, capsule shaped, film coated tablet with breakline on one side and plain on the other side.
Available in blister pack containing 10, 16, 32, 50 and 100 tablets.
Store below 30 °C in a dry place, protected from light.

NAME AND ADDRESS OF THE PRODUCT OWNER, REGISTRATION HOLDER AND MANUFACTURER
Product owner:
AFT Pharmaceuticals Ltd.
Level 1, Nielsen Building,
129 Hurstmere Road, Takapuna,
Auckland 0622, New Zealand
Product registration holder:
AFT Pharmaceuticals (S.E. Asia) Sdn Bhd
Suite 9.01-Regus, Level 9,
Menara Summit Persiaran Kewajipan USJ 1, UEP
47600 Subang Jaya Selangor, Malaysia.
Manufacturer:
Sigma Laboratories Pvt. Ltd.
6, 7, 8 Tivim Industrial Estate, Tivim,
Mapusa 403526, Goa, India

DATE OF REVISION
Dec 2019

22.5 mm

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16 mm

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16 mm

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360 mm

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Sigma Laboratories Pvt. Ltd.		
Product Name	Maxigesic 500mg/150mg Tablets	Pharma Code: 30295
Pack Size	NA	
Type	Package Insert	
Dimensions	360(L) x 140(W) mm	
Board	40 gsm bible paper	
Colours	■ Black	
Artwork Code No	PM000562-02	Superseded Artwork No. : PM000562-01
Date	06/01/2020	Country : Malaysia

Packaging Development Department		Site QA	Regulatory Affairs (Contract Giver)
Prepared by	Apporved by		
Reason for Change : 1) Product Registration Holder address changed as per Customer Requirement.			